



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض رابعياً : نغاعة رفعت بسطا حنس
المحافظة : الجيزه
الحى / المدينة : العجوزه
رقم البطاقة : 28403120101229 رقم التليفون : 01279868779
العمر (تاريخ الميلاد) : 12-03-1984 المهنة : ربه منزل
العنوان : ش نارى جرجس ارض
اللواء العجوزه الجيزه
تاريخ تسجيل الإستماره : 19-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحه-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.
تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض
بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. ----- /د. ----- /د. ----- /د. -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
جهة العلاج : -----
ملاحظات : -----
قسم : -----
نفقات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: نعاة رفعت بسطا جنس National ID: 28403120101229	
---	---

تقرير طبي

السيد
تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

DMS
Khalid

٢٠٢٤/١١
٢٠٢٤/١١/١٢٢٩
رقبة منور
النبي
مستقيمة
مترجمة
الطاقة سريية حتى
٢٠٢٤/١١/٢٣



جمهورية الكويت
مملكة الكويت
نفاذه
رفعت بسطا حسن
شماري جرجس - ارض اللواء
العجوة - العجوة



KU8595911

٢٨٤٠٣١٢٠١٠١٢٢٩

Presidential Program for Women's Health
Ministry of Health and Populationالبرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:

1- Name : نغاعة رفعت بسطا حسن

Sex:

3- Medical number:

Female

83220837

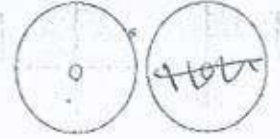
Old Med :

Old Med :

2360-2024

2-ID number:

Patient Age : 41 Y



Family history

Family History

-V0

Phone

Phone

01279868779

BMI

BMI

REASON OF MDT

REASON OF MDT
POST OPERATIVE

marital status

Marital Status

MARRIED

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

Date of MDT

24/10/2024

Co morbidity

Co Morbidity

htn

Menopausal
status

Menopausal Status

PERIMENOPAUSAL

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging :

It LIQ lesion 3.1 x2 cm , 0.2 cm away from nipple

It LIQ multiple group of microcalcifications

Mets Workup :

ct cap:free

BX

ER

PR

Her 2 Neu

SISH

Ki67

IDC

8/8

7/8

Her 2 Neu COMMENT:

SISH COMMENT:

25

BX COMMENT:

It gli with focal dcis

ER COMMENT:

PR COMMENT:

Her 2 Neu COMMENT:

-ve

SISH COMMENT:

T

N

M

2

0

0

Sonomammography

PATIENT HISTORY

LT SM+SLNB:IDC,GII with focal DCIS+focal necrosis

+ve microcalcification PT2(3CM)

LN0/8

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

1- ADJUVANT TAXOTERE ENDOXAN X4

2- ZOLADEX FOR OVARIAN FUNCTION

3- ADJUVANT ZOLADEX AI

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د. نوري / د. جميل	RADIOLOGY	RADIOLOGY د. محمد / د. نوري
SURGERY	SURGERY د. نوري	PATHOLOGY	PATHOLOGY د. نوري
Psychiatric	Psychiatric د. نوري	Nutrition	Nutrition د. نوري



Patient Name : الأستاذة / نعتاعه رفعت يسفا حسن	Request No. : 13002162
Patient No. : 28403120101229	Request Date : 10/07/2024
Path No. : SR-2357/24	Verification Date : 16/07/2024
Sex : Female	Financial Category : أورام - دار السلام - ميانره
Age : 40 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Left breast swelling.

Nature of Specimen: Tru-cut biopsy, collected at 10/7/2024.

Gross Examination:

- Multiple grayish white tissue cores up to 1.5 cm, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma with focal mucoid activity.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+3) Grade III.
- * Ductal carcinoma in situ: Focally positive.
- * Perineural invasion: Negative.
- * Angioinvasion: Negative.

Diagnosis:

Left breast swelling, Tru-cut biopsy:

Invasive duct carcinoma, Grade III, with focal mucoid activity and focal DCIS.

For further IHC (ER, PR, Her2, Ki67).

Signature

Prof. Dr. Lubna Omar.

Dr. Esraa Magdy.

Verified by :



Patient Name : الأستلاءه / نعنائه رفعت بسطا جنس :	Request No. : 13003721
Patient No. : 28403120101229	Request Date : 26/09/2024
Path No. : 3648/24	Verification Date : 12/10/2024
Sex : Female	Financial Category : اورام دار السلام - مبادره
Age : 40 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, Up-Front.
Nature Specimen: Left simple mastectomy and SLNs.

Gross Examination:

- * Procedure: Left simple mastectomy specimen measured 19x15x4 cm with covering skin ellipse measured 14x5 cm.
- * Tumor extent: A greyish white tumor bed site.
- * Size: 3.5x2.3x2cm
- * Skin involvement: Unremarkable grossly.
- * Distance from margins: 1.2 cm away from deep margin and 1.5cm away from the skin
- * Lymph nodes number and size: Multiple fibrofatty tissue peice measured 2.5x2.5x2.5 entangling 8 LNs up to 2x1 cm.
- * Rest of breast tissue show glistening rubbery cut section.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1)
- * Ductal carcinoma in situ: Focal (40%) (Commedo & Non comedo)
- * Skin involvement: Free nipple and areola.
- * Lymph node involvement: Free SLNs frozen (0/5), Free paraffine SLNs (0/3).
- * Resection Margins: Free deep margin.
- * Microcalcifications: Positive.
- * Additional pathologic findings: Focal necrosis.

Frozen section results:

Free SLNs (0/5).

Diagnosis:

Left breast, simple mastectomy and SLNs Biopsy:

Verified by :

Dr. Sagda El-Kady

Printing Date : 14/10/2024 13:16:47

Page 1 of 2

Printing By : .marwa mohamed youss

Patient Name : الأستاذة / نعناعه رفعت بسطا جنس :	Request No. : 13003721
Patient No. : 28403120101229	Request Date : 26/09/2024
Path No. : 3648/24	Verification Date : 12/10/2024
Sex : Female	Financial Category : اورام دار السلام - مبادره
Age : 40 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

Invasive duct carcinoma, Grade II with focal DCIS, Focal necrosis and positive microcalcification.

Free deep margin.

Free nipple and areola.

Free SLN (frozen) 0/5 , Free SLN (paraffine) 0/3

Total Nodal status : Free SLNs (0/8).

- Stage: pT2, pN0

Signature

Dr. Marwa Mahmoud.

Dr. Sagda Elkady.

Verified by :

Dr. Sagda El-Kady

Printing Date : 14/10/2024 13:16:47

Page 2 of 2

Printing By : .marwa mohamed youss

CAP
ACCREDITED

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name :
Patient No. :
Path No. :
Sex :
Age :
Referring Doc. :

Request No. :
Request Date :
Verification Date :
Financial Category :
Department :
IHC No. :
Registered by :

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast swelling.

Nature of Specimen: Tru-cut biopsy, collected at 10/7/2024.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity).
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Moderate nuclear reaction in 90% of tumor cells (score 7/8).
- Her2: No membrane reaction in the tumor cells (score 0).
- Ki67: 25% nuclear reaction of tumor cells.

Conclusion:

Left breast swelling, Tru-cut biopsy:

Invasive duct carcinoma, Grade III, with focal mucoid activity and focal DCIS.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 7/8).
- Her2: Negative (score 0).
- Ki67: Positive in 25% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1,

Verified by :

Printing Date : 23/07/2024 12:46:24

Page 1 of 2

Printed By : Mrs. Mayada Hegazy



Patient Name: الأستاذة / مناعة وديعة مصطفى حسن
Patient No.: 28403120101229
Path No.: SR-2357/24
Sex: Female
Age: 40 Y
Referring Doc.

Request No.: 13002162
Request Date: 10/07/2024
Verification Date: 23/07/2024
Financial Category: اورام دار السلام - ميانو
Department: Out Patient
Registered by: marwa mohamed youssef

IMMUNOHISTOCHEMISTRY REPORT

>1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof. Dr. Lubna Omar.
Dr. Menna Nabil.

Verified by:

Printing Date: 23/07/2024 12:46:24

Page 2 of 2

Printed By: Mrs. Mayada Hegazy



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Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Min El Qadima - Cairo, Egypt

PATIENT NAME: NIENAEUH REFAT BASTA HANES

GENDER: Female

DATE : 01-Jul-2024 08:55 AM

PATIENT ID: 10124244273

AGE: 40 y

ORDER: 2913395,2913396

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- Left breast mass.

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound

REPORT:

- Heterogenous dense breasts which may obscure small lesions (ACR C).

Left breast:

- Left LIQ obscured lesion measures about 3.1x2.4 cm, 2.2 cm away from the nipple (measured in MLO view).
- Overlying left LIQ multiple groups of microcalcifications extending deeply, seen over an area of about 7x5 cm, about 1.7 cm away from the nipple (measured in CC view).
- Corresponding by US to left LIQ 7:00 ill defined hypoechoic lesion measures about 2.4x1.6 cm, 2.5 mm away from the overlying skin. Surrounding prominent ducts are seen reaching retroareolar and showing echogenic content. No vascularity detected on color Doppler application.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- No detected pathological axillary lymph nodes.
- No detected pathological supra or infraclavicular lymph nodes.

Right breast:

- No spiculated mass lesions or grouped microcalcifications.
- Complementary Ultrasound showed no solid or cystic mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Right nonspecific axillary lymph nodes.

OPINION:

PATIENT: NIENAEUH REFAT BASTA HANES, 10124244273, Female, 40 y
Report version 1, 01-Jul-2024 09:26 AM



- most malignant looking lesion with related suspicious LIQ multiple groups of microcalcifications for histopathological assessment (BIRADS 5).
- Bilateral mammary adenosis (BIRADS 2).

RECOMMENDATION:

Referral to clinic

Reported By,

Dr. Lamiaa Abdallah, MSc

01-Jul-2024 09:25 AM

PATIENT: NIENAEUH REFAT BASTA HANES, 10124244273, Female, 40 y
Report version 1, 01-Jul-2024 09:26 AM

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16602

مركز باهية اليوم
و شارع غلوة و شارع الهرم بحول داري، الدقة



مبنى دة الشيخ زائد
L الى اولى الشيخ و دة 24 اوت 2024



Immunohistochemistry
[[DOCTYPE:IHC]]



Women's Health Outreach Program

Name: ne3naa Refaat yastta 40

Date: 3-7-2024

Full field Bilateral Digital Mammography & ultrasonography

Clinical summary left breast palpable lump from one week

Detailed Findings

Technique

Craniocaudally & Medio lateral oblique views

High resolution ultrasound

Left breast

- Dense glandular mammary parenchyma ACR c
- LIO regional micro calcifications seen in area about 7x2 cm away from nipple by 2 cm ,with underlying partially defined irregular round dense lesion measures about 2.5x2cm
- Complementary US corresponding to mammo finding between 8-9 o'clock shows irregular inhomogeneous hypoechoic solid lesion measures 30x20mm away from nipple by 2cm and away from skin by 2mm
- Multiple enlarged axillary nodes seen at level one with preserved fatty hilum and cortical thickening not exceeded 3mm ,at level II enlarged lymph node measures 12x8mm with cortical thickening 4mm

Right breast

- Fairly dense glandular mammary parenchyma ACR density c
- No masses density ,asymmetry or suspicious clusters of microcalcifications.
- Ultrasound demonstrated mottling parenchymal appearance
- No solid or cystic lesion
- Preserved axillary LNs .

Conclusion:

Left breast (BIRADS5) malignant looking solid lesions with regional micro calcification for biopsy

Right breast: (BIRADS 1).

DR Marwa M.Hassan

PATIENT NAME: NIENAEUH REFAT BASTA HANES
GENDER: Female

DATE : 31-Jul-2024 02:02 PM

PATIENT ID: 10124244273

AGE: 40 y

ORDER: 2968474,2968475

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH TRIPHASIC STUDY

TECHNIQUE:

- Multislice helical CT acquisition with automatic IV low osmolar non-ionic contrast medium infusion 50 ml.

FINDINGS:

1-CHEST

- Left breast lower inner quadrant irregular heterogenously enhancing mass lesion is seen measuring about 3.3x 2.7 cm along its maximum axial dimensions.
- No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- Two bilateral pulmonary small calcified nodules, likely post granulomatous sequel.
- No CT evidence of pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- No pericardial effusion detected.
- Small hiatus hernia.

2- ABDOMEN AND PELVIS

- The liver is enlarged showing homogenous reduced parenchymal attenuation denoting fatty infiltration. No pathologically enhancing focal lesions noted throughout different study phases.
- Prominent common hepatic and common bile duct with CBD measuring about 8 mm. Slightly prominent pancreatic duct.
- No intra-hepatic biliary radicles dilatation.
- Patent homogenously enhancing portal vein and its main branches.
- Bilateral adnexal small cysts, for US correlation.
- Normal appearance of the pancreas, spleen, kidneys (right renal tiny gravel and few bilateral renal cortical cyst, the largest on the left),

PATIENT: NIENAEUH REFAT BASTA HANES, 10124244273, Female, 40 y
Report version 4, 31-Jul-2024 03:20 PM



- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
 - No pathologically enlarged abdominal or pelvic lymph nodes.
 - No abdominal or pelvic masses or collections.
 - **The revised bone window revealed:**
 1. No CT evidence of definite osseous lytic or sclerotic lesion.
 2. No CT evidence of vertebral collapse or extra-osseous soft tissue component.
 3. Left femoral subcortical pseudocystic changes, likely degenerative.
- Opinion:**
- Left breast malignant process, for histopathological correlation.
 - No definite pulmonary or osseous metastatic deposits.
 - Fatty hepatomegaly with no CT evidence of hepatic metastatic deposits.

Reported By,
Dr. Nouran Atrass, MSc
Dr. Mennatallah Mohamed, MD

31-Jul-2024 03:20 PM

PATIENT: NIENAEUH REFAT BASTA HANES, 10124244273, Female, 40 y
Report version 4, 31-Jul-2024 03:20 PM

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د. نورا أتراس
د. منة الله محمد



د. منة الله محمد
د. نورا أتراس

